SSA EQUALITY IMPACT AND NEEDS ANALYSIS

Directorate	Adult Social Care and Public Health
Service Area	Public Health
Service/policy/function being assessed	Mental Health Action Plan
Which borough (s) does the service/policy apply to	Wandsworth
Staff involved in developing this EINA	Graeme Markwell
Date approved by Directorate Equality Group (if applicable)	**N/A**
Date approved by Policy and Review Manager All EINAs must be signed off by the Policy and Review Manager	Jamie Fisher. 27th October 2025
Date submitted to Directors' Board	14th October 2025

1. **Summary**

Please summarise the key findings of the EINA.

Introduction

Mental health is a corporate plan priority for Wandsworth Council. The Public Mental Health Action Plan (PMHAP) is to be delivered through a range of Council led programmes, supported by evidence-based recommendations set out in the Wandsworth Mental Health Needs Assessment, 2022 (MHNA). The PMHAP aligns with the priorities set out in the Council's Corporate Plan to ensure that Wandsworth is a fair, compassionate, and sustainable place to live.

The MNHA identifies clearly that the mental health of Wandsworth residents has deteriorated, the evidence of which can be seen in national surveys such as the Mental Health of Children and Young People and the Adult Psychiatric morbidity Survey. These risks to mental health and the associated poor outcomes do not fall evenly across the population. People living in poverty; those with physical disabilities and long-term conditions; people with neurodevelopmental conditions; children in care; people from Global Majority groups; and LGBTQ+ people all experience much poorer mental health outcomes because of intersecting disadvantages and discrimination. In addition, the life expectancy of a person with a severe mental illness is about 20 years shorter than someone without a diagnosis, and the gap is getting bigger.

The PMHAP will target groups at higher risk of mental disorder through a range of council programmes. It is anticipated that the plan will reduce health inequalities for vulnerable groups, including those with protected characteristics, by providing a range of services to reduce risk factors and enhance protective factors.

Positive Impacts

The PMHAP will also use the Metal Well-being Impact Assessment (MWIA) Tool to enhance mental well-being by evaluating the impact of policy, services, and community initiatives on resident's mental health and well-being. MWIA helps identify both positive and negative impacts of a project or proposal on mental well-being. It considers factors that can enhance mental health as well as those that might impact negatively. It facilitates co-production and co-design of interventions and services,

ensuring alignment with community needs with a focus on communities with vulnerabilities including those with protected characteristics.

Negative Impacts
While the PMHAP should not have any negative impact on the outcomes for any group in the population, there are service areas where data is not available to understand the impact on people with protected characteristics. This may result in a missed opportunity to address inequalities they may experience arising from the actions in the strategy.

Key points identified through the EINA.

Children and Young People

- Increased prevalence and incidence of common mental health disorders.
- Increased prevalence, complexity, acuity and longevity of mental health conditions, especially self-harm and eating disorders, particularly since the COVID-19 pandemic.
- Rising demands on mental health services are leading to increased wait times and insufficient care.
- The most common reasons for referral to services are anxiety and neurodevelopmental (excluding Autistic Spectrum Disorder).
- A decrease in referrals to most services during 2020/21, associated with the COVID-19 pandemic. Followed by a surge in need and an increase in waiting times for services.
- A noticeable increase in eating disorders in girls.
- Significant unmet need in children and young people require a single professional response (Tier 2 services).
- A surge in young people requiring crisis care during the height of pandemic restrictions.
- Increased levels of inpatient and crisis care, disproportionately affecting girls.
- A high prevalence of children with Special Educational Needs in schools with social and emotional needs as the primary issue.
- Schools are struggling to cope with pupils increased mental health needs.
- Black, Asian and minority ethnic and LGBTQ+ children and young people are particularly vulnerable groups.

Adults and Older People

- One in five adults meet the criteria for a common mental disorder.
- Approximately half of people with a common mental disorder reported current use of mental health services.
- Increasing prevalence of mental health conditions, particularly severe anxiety and depression, trauma and complex needs, particularly since the COVID-19 pandemic and cost of living crisis.
- Mental health disorders are more common in people living alone, in poor physical health or with a disability or who are unemployed.
- Service users with neurodiversity and/or learning disabilities, those with co-occurring mental health and substance misuse disorders, carers and Black, Asian and minority ethnic groups are particularly vulnerable.
- Significant barriers to accessing and receiving mental health services including long waiting lists, limited time available for support and high thresholds for acceptance.
- Women are more likely than men to report symptoms of common mental disorder (CMD).
- Women are more likely to access mental health services than men.

- Young women have emerged as a high-risk group with higher rates of CMD, self-harm and Post Traumatic Stress Disorder.
- Parents and carers, people who are unemployed and lower income families, young adults, people with existing conditions and ethnic minorities have experienced increased prevalence of CMD due to the pandemic.
- A drop in referrals across most treatment services during 2020/21.
- 25-34-year-olds are the most common age group in services.
- Ethnic minorities are represented proportionately in psychological therapies and community services.
- Ethnic minorities are overrepresented in inpatient services.
- The Adult Social Care Mental Health Team experienced a surge in referrals during 2021/22.
- There has been a steady increase in numbers of residents assessed as having mental health care and support needs.
- More men than women require social care for mental health needs.
- Minority ethnic groups are under-represented in Home Treatment and Crisis Intervention Teams.

2. Evidence gathering and engagement

a. What evidence has been used for this assessment? For example, national data, local data via DataRich or DataWand

Evidence	Source
Evidence of need based on epidemiological data and corporate input from stakeholders.	Wandsworth Mental Health Needs Assessment Mental Health of Children and Young People in England, 2023 - wave 4 follow up to the 2017 survey - NHS England Digital Adult Psychiatric Morbidity Survey: Survey of Mental Health and Wellbeing, England, 2023/4 - NHS England Digital
Demographic data	DataWand
Census data 2021 Wandsworth	PowerPoint Presentation (datawand.info)
Mental Health and Physical Disabilities Study	Mental health problems among adolescents and young adults with childhood-onset physical disabilities: A scoping review - PMC (nih.gov) CBP-9602.pdf (parliament.uk)
Mental Health and Gender Study	Gender-related self-reported mental health inequalities in primary care in England: a cross-sectional analysis using the GP Patient Survey - The Lancet Public Health
Ethnicity and Mental Health in Children and Young People	Associations between ethnicity and mental health problems among children and adolescents in the United Kingdom: A systematic review and narrative synthesis | BMC Public Health | Full Text

Director of Public Health, Annual Public Health Report. Enough is Enough.	Director of Public Health Annual Report 2023 (wandsworth.gov.uk)

b. Who have you engaged and consulted with as part of your assessment?

Individuals/Groups	Consultation/Engagement results	Date	What changed as a result of the consultation
Mental Health Review Workshop	Consultation regarding strategic vision and priorities	26/02/24	Consultation supported the development of tertiary prevention priorities and contributed to strategic vision.
Wandsworth Older People's Forum	As above	12/03/24	Consultation supported the development of prevention priorities for older people. Participants contributed to strategic vision.
Wandsworth Mental Health Partnership (Health, social care, VCS, service users and carers).	As above	23/09/25	Consultation supported the development of prevention priorities for adults. Participants contributed to strategic vision.
Wandsworth Childrens Mental Health Partnership (Health, social care, Children's Services, VCS).	As above	10/09/25	Consultation supported the development of prevention priorities for children and young people. Participants contributed to strategic vision.
Sound Minds Service User Group	As above	30/04/24	Consultation supported the development of tertiary prevention priorities and contributed to strategic vision
Black Minds Matter Youth Group	As above	02/11/23	Consultation supported the development of prevention priorities for children and young people from ethnic minority communities. Participants contributed to strategic vision.
BME Mental Health Forum	As above	28/03/24	Consultation supported the development of prevention priorities for ethnic minority communities. Participants contributed to strategic vision.
Wandsworth Co-production Group	As above	18/03/24	Consultation supported the development of secondary and tertiary prevention

			priorities for service users. Participants contributed to strategic vision.
Public Mental Health Strategy Multi-agency Workshops	As above	25/03/24	Consultation supported the development of primary, secondary and tertiary prevention priorities for service users. Participants contributed to strategic vision.

3. Analysis of need

Protected group	Findings
Age	**Demographics** The table below shows the population age structure for Wandsworth. Wandsworth has one of the youngest populations in the country and by 2029, the boroughs population will rise to 373,000 with the biggest increase within the 20–39-year-old age group. 1 in 10 residents are 65 years old or older, by 2031 this population age group will increase by 26%.

Age Group (5 Year)	Wandsworth	Inner London	London	Population
0 to 4	6.00%	5.42%	5.98%	19,631
5 to 9	5.47%	5.10%	5.90%	17,903
10 to 14	5.26%	5.21%	6.03%	17,193
15 to 19	4.52%	5.06%	5.57%	14,767
20 to 24	5.21%	6.50%	5.65%	17,047
25 to 29	11.53%	10.94%	8.53%	37,712
30 to 34	11.60%	11.37%	8.60%	37,931
35 to 39	10.70%	9.86%	8.67%	34,989
40 to 44	8.43%	8.04%	7.89%	27,563
45 to 49	6.44%	6.49%	6.73%	21,071
50 to 54	5.76%	5.98%	6.37%	18,849
55 to 59	5.03%	5.42%	5.91%	16,457
60 to 64	3.84%	4.27%	4.83%	12,542
65 to 69	3.00%	3.24%	3.76%	9,797
70 to 74	2.54%	2.55%	3.11%	8,321
75 to 79	1.95%	1.93%	2.47%	6,474
80 to 84	1.36%	1.30%	1.66%	4,519
85+	1.30%	1.31%	1.73%	4243

Children and Young People – Mental Health Need

- There are an estimated 6,110 5-19-year-olds with a mental health disorder in Wandsworth.

	• Emotional disorders have become more common, with an increase in both boys and girls, while other disorder types were stable. • Anxiety disorders are the most comment type of emotional disorders across all age groups. • The highest number of emotional disorders is among those aged 11-16. • Rates of mental disorder were higher in older age groups: young people between 17-19 were three times more likely to have a disorder than those aged 5-17. **Children and Young People – Service Data** • Between 2018 and 2021 the number of children and young people referred to the Single Point of Access (SPA) increased each year. Most referrals are from those between 11-16 years. • The most common reasons for referral to SPA were being in crisis (26%) and anxiety (17%). • There was a surge in young people requiring crisis care during the height of pandemic restrictions. • Between 2018 and 2022 referrals to the SPA for self-harm increased each year. • In 2021/22 (Q1-3) there were increased numbers of referrals to the Adolescent Assertive Outreach Team, Children and Adolescent Mental Health Services (CAMHS) Emergency Care Service and CAMHS Eating Disorder Service. This is likely due to an increase in acute mental health crisis since the COVID-19 pandemic. • Between 2018/19 and 2020/21, the number of children and young people (CYP) referred to CAMHS Tier 3 declined annually. • Increased waiting times for services in 2021/22 (Q1-3) suggests a surge in need. **Adults and Older People – Mental Health Need** • There are an estimated 45,877 adults in Wandsworth with a common mental health disorder (CMD) • It is estimated that 3,709 adults in Wandsworth have a Serious Mental Illness (SMI). • It is estimated that one in five adults in Wandsworth have a mental disorder. • Primary care data in Wandsworth shows that depression is more common than anxiety in adults. • In 2019 self-harm was the leading cause of death for males and females between the ages of 15-49. The trend for self-harm admissions has been stable over the last decade in Wandsworth. • The prevalence of most CMDs and SMIs diminishes with age. • The prevalence of dementia in Wandsworth is increasing faster than average elsewhere, by 2035 those <65 years with dementia is expected to increase by 47%. **Adults and Older People – Service Data** • The number of service users referred to Talk Wandsworth (part of Improving Access to Psychological Therapies (IAPT)) and voluntary sector services decreased in 2020/21, likely due to the impact of the COVID-19 pandemic.

- Referrals to the Single Point of Contact, Community Mental Health Teams (CMHT) have remained stable from 2018/19 to 2020/21 though the number of referrals accepted has declined each year.
- There was an increase in referrals to the Early Intervention Teams in 2020/21 with highest rate of referrals for those aged 18-24.
- In 2020/21 the number of service users referred to Crisis, Inpatient services and Adult Eating Disorder services declined from previous years.
- There was a large drop in the number of service users referred to the Liaison Psychiatry Service from the Emergency Department in 2020/21, which was likely caused by reduced attendance to ED during the COVID-19 pandemic.
- Across all older adult mental health services there were reduced rates of referrals in 2020/21, likely due to the COVID-19 pandemic.
- Despite the decreasing number of referrals, the wait times for Memory Assessment Services and Older People's CMHT have increased.
- Most services show a general trend of a declining rate of referral with age, with the highest representation seen among the 18-24 age group and the lowest among those aged 75+.
- Consistently across services the highest rate of referrals is among those aged 18-24.

Disability

Demographics.

The tables below show the proportion of the population in Wandsworth with a health problem or disability overall, and in 5- year age bands. Around 10% of the population experience limitations on day-to-day activities due to disability. The groups with the highest proportions are those aged 25 to 34 years of age reflecting the population age structure of the borough.

Age	Population
0 to 4	7.18%
5 to 9	4.83%
10 to 14	4.15%
15 to 19	3.81%
20 to 24	7.49%
25 to 29	15.52%
30 to 34	13.73%
35 to 39	9.69%
40 to 44	7.37%
45 to 49	5.90%
50 to 54	4.60%
55 to 59	3.71%

Health Problem/Disability	Population
Day-to-day activities not limited	89.32%
Day-to-day activities limited	10.68%

Learning Disabilities in Children and Young People – Mental Health Need

- In 2024/25 there were 2,202 boys (3-19 years) with an Education and Health Care Plan (EHCP)
- In 2024/25, there were 973 girls (3-19 years) with an EHCP.
- In 2024/25, there were 3,175 CYP (3-19 years) with an EHCP:

- Estimated 1,500 with a probable mental health disorder.

Learning Disabilities in Adults – Mental Health Need

- Estimated 1,343 people with learning disabilities (16-64 years) with a mental health disorder.
- Estimated 179 people with learning disabilities (65+ years) with a mental health disorder.

Physical Disabilities in Children and Young People – Mental Health Need
This international research shows that mental health problems are common in adolescents and young adults with childhood-onset physical disabilities ranging from 13.6% to 53.0% of those surveyed. The three most common mental health concerns investigated were depression and mood-related difficulties, anxiety, and social and behavioural difficulties.Mental health problems among adolescents and young adults with childhood-onset physical disabilities: A scoping review - PMC

Physical Disabilities in Adults – Mental Health Need
In 2021/22, the most common impairment types among working age adults were mental health (reported by 44% of disabled people in this age group) and mobility (43%). illness, in line with the Government Statistical Service's (GSS) Impairment harmonised standard. Source: Family Resources Survey 2021/22 – GOV.UK

Sex

Demographics
The table below shows the sex distribution of the Wandsworth Population. The borough's population is made up of 52% females and 48% males. The proportion of women and men are roughly equal across the life-course age-bands until later in life. As women experience longer life expectancy than men, 84.2 years in females versus 80.6 years in males, by the time people are aged 75 years and over, one starts to see a shift in balance between the proportion of both genders (59% female, 41% male).

Sex	Population
Female	170,345
Male	156,665

Children and Young People - Mental Health Need (Sex)
The table below shows the estimated number of children and young people (CYP) in Wandsworth with emotional disorders by age and sex, based on national prevalence.

	5-10 year-olds			11-16 year-olds			17-19 year-olds	
Type of Disorder	Boys	Girls	All	Boys	Girls	All	Boys	Girls
Emotional Disorders Overall	558	413	971	645	953	1599	301	858
Anxiety Disorders	533	391	924	566	847	1414	293	778
Depressive Disorders	44	27	71	149	336	486	122	250
Bipolar Affective Disorder	Data Not Available	<5	Not Available	<5	Not Available	10		

- Over the life course of CYP, it is predicted that there will be a shift from a higher prevalence of mental disorders in males to females.
- It is estimated that the prevalence of mental disorders will be highest among boys between the ages of 5–10 years.
- Between the ages of 11–16 years the prevalence of mental disorders is estimated to be relatively equal between males and females.
- There is estimated to be a higher prevalence of mental disorders in girls aged of 17–19 years.
- It is estimated that there will be a higher prevalence of behavioural, hyperactivity and other less common disorders among boys.
- By contrast, it is estimated that girls will be more likely to experience emotional, anxiety and depressive disorders.

Children and Young People – Service data (Sex)

- Between 2018/19 and 2021/22 (Q1-3), the sex balance of CYP referred to the CAMHS Single Point of Access (SPA) shifted. In 2018/19 and 2019/20, there were more males (55% and 53% respectively) than females (45% and 47% respectively) referred to the SPA. However, in 2020/21 and 2021/22 (Q1-3), there were more females (53% and 54% respectively) referred than males (47% and 46% respectively).
- The female referral rate increased from 3,223 per 100,000 in 2018/19 to 3,686 per 100,000 in 2019/20, and 4,243 per 100,000 in 2020/21. The rate of female referrals in Q1-3 of 2021/22 was already at 4,231 per 100,000.
- The male referral rate increased between 2018/19 and 2019/20 from 3,811 per 100,000 to 4,015 per 100,000. However, it fell to 3,596 per 100,000 in 2020/21 and was 3,429 per 100,000 in 2021/22 (Q1-3)
- In 2021/22 (Q1-3) there were increased numbers of referrals to the Adolescent Assertive Outreach Team, CAMHS Emergency Care Service and CAMHS Eating Disorder Service. This is likely due to an increase in acute mental health crisis since the COVID-19 pandemic. From 2019/20 there was a signifyingly higher proportion of females referred to all three services.
- There has been a noticeable increase in eating disorders in girls. Between 2019/20 and 2020/21 there was a 68% increase of females referred to the Eating Disorder Service

- There has been a noticeable increase in eating disorders in girls. Between 2019/20 and 2020/21 there was a 68% increase of females referred to the Eating Disorder Service.
- Young women in Wandsworth have emerged as a high-risk group.

Adults and Older People – Mental Health Need (Sex)
The table below able shows the estimated number of people in Wandsworth with a common mental disorder in the past week by sex and type, using ONS mid-2020 population data.

Type of Common Mental Disorder	Sex Males	Females
Generalised Anxiety Disorder	6242	9688
Depressive Episode	3694	5271
Phobias	2293	4274
Obsessive Compulsive Disorder	1401	2137
Panic Disorder	382	1140
Common Mental Disorder - Not Otherwise Specified	7389	13677
Any Common Mental Disorder	16816	29491

- Women in Wandsworth are more likely to have a CMD than men.
- Using national prevalence data, it is estimated that more women than men in Wandsworth have a psychotic disorder.

The table below shows the estimated number of people in Wandsworth with a psychotic disorder by sex, based on national prevalence.

Sex	Estimated Number of Adults
Men	892
Women	1,140
All Adults	1,889

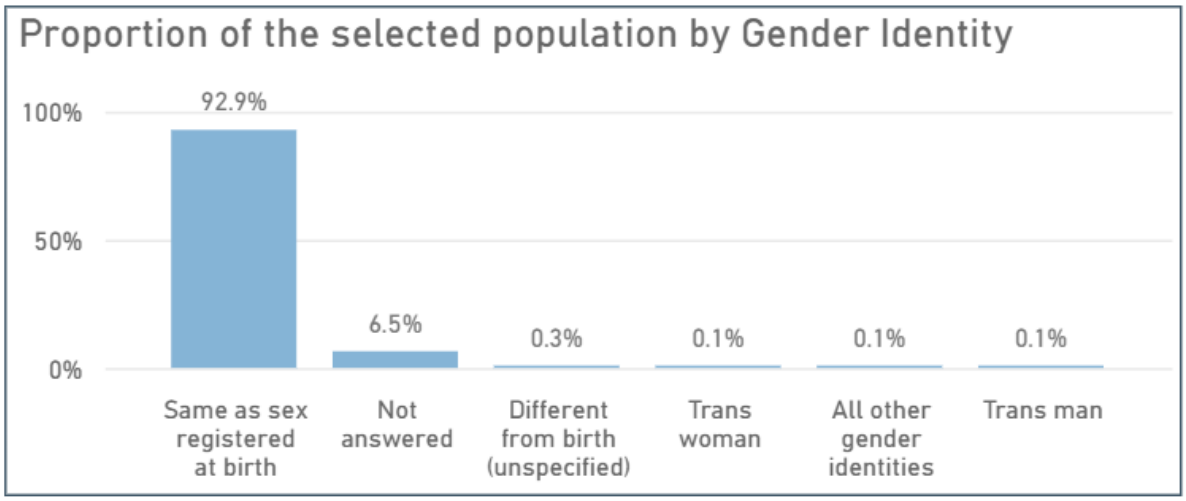
- For women in Wandsworth, those between 16-24 were more likely to screen positive for PTSD.
- For men, the rate of a positive screen for PTSD remained stable from 16–65, before declining with old age
- It is estimated that antisocial personality disorder is more common in men, while borderline personality disorder is more common in women.

Adults and Older People – Service Data (Sex)
- Between 2018/19 and 2021/22 (Q1-3), there were consistently more females than males referred to the SPA (57% females vs. 43% males).
- Between 2018/19 and 2021/22 (Q1-3), more females (av. 58%) than males (av. 42%) were referred to the CMHT.
- Most services consistently receive a greater number of referrals from females than males at a relatively consistent rate. The services where this is not apparent are the Adult Eating Disorder Service of which over 90% are female;

the referrals to Psychiatric Intensive Care Unit (PICU) where around 80% are males; and in Adult Social Care where most service users are male.

- In 2018/19 and 2019/20, there were more males (57% and 60%) referred to the Early Intervention Teams than females (43% and 40%).
- This is a higher proportion of males than we would expect from national data on psychosis.
- The balance of males and females referred to the service was relatively equal in 2020/21 and 2021/22 (Q1-3), which fits to national data on the prevalence of psychosis by sex.
- This was caused by an increase in the rate of female referrals over these years, and a simultaneous decrease in the rate of male referral.

Gender reassignment



Source: Census 2021: Gender Identity, Wandsworth (16+ population), DataWand

Mental Health Need – Gender Identity.

- A nationally representative study in England found that transgender people are more likely to have long-term mental health conditions compared to cisgender individuals.
- The risk of a long-term mental health condition in England is approximately:
  - Transgender men: 16.4%
  - Transgender women: 15.9%
  - Cisgender men: 8.8%
  - Cisgender women: 12%
  - Non-binary transgender people: Nearly 47.2%

Gender Identity – Service Use

The Wandsworth Mental Health Needs Assessment did not provide service use data regarding gender identity.

Marriage and civil partnership

The tables below show marital status in Wandsworth categorised by age. There are no specific findings from the MHNA in relation to marriage and civil partnerships.

Marital Status	Population

Single (never married or never registered a same-sex civil partnership)	54.21%
Married	32.59%
Divorced or formerly in a same-sex civil partnership which is now legally dissolved	6.46%
Widowed or surviving partner from a same-sex civil partnership	3.69%
Separated (but still legally married or still legally in a same-sex civil partnership)	2.47%
In a registered same-sex civil partnership	0.59%

Pregnancy and maternity

Mental Health Need – Pregnancy and Maternity

It is expected that perinatal mental health problems affect up to 20% of new or expectant mothers. Perinatal mental health problems can have long-lasting effects on the woman and her family, as well as on the development of the child.

There was 3,850 live births in 2024 The prevalence of Post Natal Depression is estimated to be between 10-15%. It is estimated that in 2024 385 – 580 women in Wandsworth experienced Post Natal Depression.

Race/ethnicity

Demographics

The table below shows the ethnic groups in Wandsworth. Wandsworth is an ethnically diverse Borough when compared to England, however, compared to London and Inner London, Wandsworth has much higher proportion of White British.

Ethnicity	Population (%)
White	69.92
Black	10.77
Asian	10.23
Mixed	6.3
Other	2.78

Table 2: Ethnicity Breakdown, Numbers and Percentage, 2019, Wandsworth, Inner London, and Londor

Ethnicity	Wandsworth n	Wandsworth %	Inner London %	London %
White	230621	70.2	56.9	56.6
White British	157737	48	34.7	39
White Irish	9484	2.9	2.3	2
White Other	63400	19.3	19.9	15.6
BAME	98209	29.9	43.1	43.3
Black Caribbean	11802	3.6	4.5	3.8
Black African	16999	5.2	8	7.2
Pakistani	10559	3.2	1.9	3
Indian	8322	2.5	3.6	7.1
Other BAME	50527	15.4	25.1	22.2
Total	**328830**	**100**	**100**	**100**

Source: GLA Housing-led ethnic group projections

Children and Young People - Mental Health Need (Ethnicity)

The table below shows the estimated number of children in Wandsworth aged 5 – 19 years with any mental disorder by ethnicity and sex, based on national prevalence data.

Ethnicity	All	Boys	Girls
White British	2582	1275	1308
White Other	273	147	126
Black/Black British	430	131	295
Asian/Asian British	292	110	183
Mixed/Other	673	347	282

General Prevalence Across Ethnic Groups:

- The overall prevalence of mental health disorders is broadly similar across most ethnic groups, but there are notable differences in the types of disorders experienced.

Anxiety and Depression:

- Pakistani, Bangladeshi, and Black Caribbean children show a higher risk of anxiety and depression compared to White British children.
- These may be influenced by socioeconomic factors, cultural stigma, and access to mental health services.

Conduct Disorders and hyperactivity):

- Black Caribbean children are more likely to experience externalising and conduct problems.
- Pakistani and Bangladeshi children show similar rates of externalising disorders to White British children.

Lower Risk in Some Groups:

- Black African children are less likely to develop general mental health problems.

Socioeconomic and Cultural Influences:

- The survey highlights that ethnicity alone is not a sufficient predictor of mental health outcomes.
- Factors such as income, parental mental health, family dynamics, and community support play significant roles

Source: Associations between ethnicity and mental health problems among children and adolescents in the United Kingdom: A systematic review and narrative synthesis | BMC Public Health | Full Text

Adults and Older People – Mental health Need (Ethnicity)

The table below shows the estimated number of people in Wandsworth with a common mental disorder in the past week by ethnicity and sex, using ONS mid-2020 population data.

Sex	Ethnic Group				
	White British	White Other	Black/Black British	Asian/Asian British	Mixed, multiple and other
Males	9184	3053	1492	1568	988
Females	14350	4155	4300	3066	1946
All Adults	23646	7195	5850	4478	2890

- It is estimated that most adults with a CMD in Wandsworth will be from White British or White Other ethnic groups.
- Across all ethnic groups, it is estimated that more females than males will experience any type of CMD.
- It is estimated that there will be a particularly high number of females compared to males with a CMD in Black or Black British ethnic groups; it is estimated that there will be 5850 Black or Black British adults with a CMD in Wandsworth, of which 4300 are expected to be female.
- Rates of psychotic disorder were found to be higher in Black men (3.2%) than men from other ethnic groups.

Religion and belief, including non belief

The table below shows religion, belief, and non-belief in Wandsworth. There were no specific findings in relation to the priority areas.

Religion	Population
Christian	52.96%
No religion	26.95%
Muslim	8.06%
Religion not stated	7.86%
Hindu	2.12%
Buddhist	0.84%
Jewish	0.53%
Other religion	0.42%
Sikh	0.27%

Sexual orientation

Children and Young People Mental Health Need – Sexual Orientation

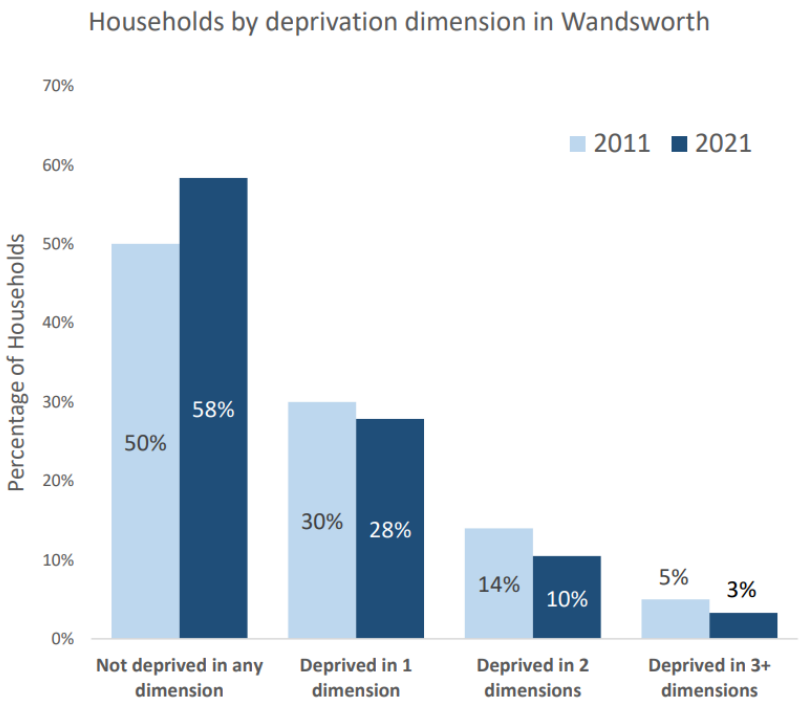
It is estimated that in Wandsworth there are:

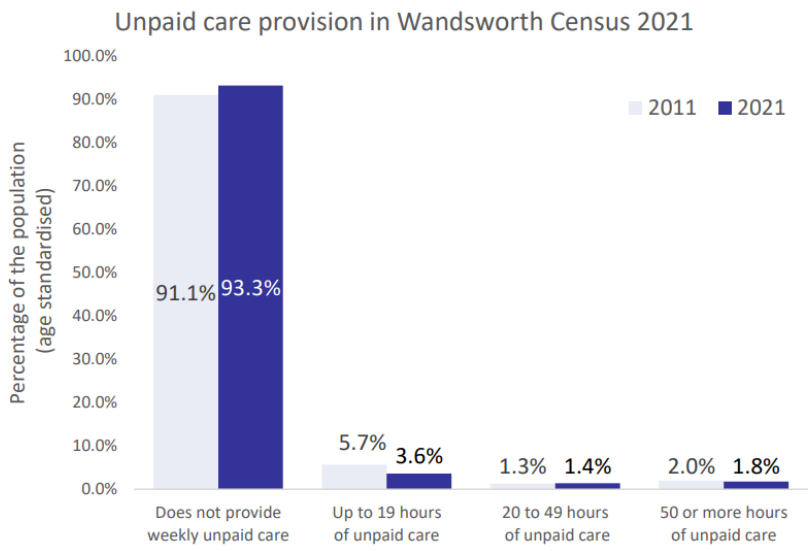
- Estimated 409 non-heterosexual girls (14- 19 years) with a mental health disorder.
- Estimated 225 non-heterosexual boys (14-19 years) with a mental health disorder.

Adult Mental Health Need – Sexual Orientation

Estimated LGB mid-2014 population of England 5.3% (18 – 90 yrs. old):

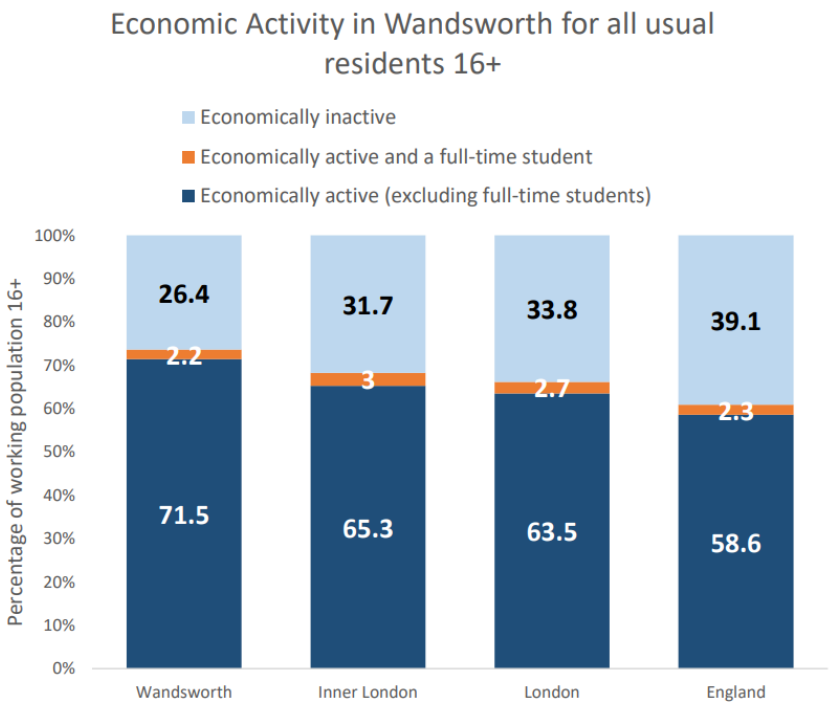
- Estimated 13,953 LGB people in Wandsworth.
- Estimated 3,098 LGB people with a neurotic disorder in the past week.
- Estimated 879 LGB people with generalised anxiety disorder.
- Estimated 572 LGB people experienced a depressive episode in the last week.

	• Estimated 2,874 LGB people have experienced suicidal thoughts. • Estimated 1,242 LGB people attempting suicide.
Across groups i.e older LGBT service users or Black, Asian & Minority Ethnic young men.	No specific data was available across groups.
Socio-economic status (to be treated as a protected characteristic under Section 1 of the Equality Act 2010) Include the following groups: • **Deprivation (measured by the 2019 English Indices of Deprivation)** • **Low-income groups & employment** • **Carers** • **Care experienced people** • **Single parents** • **Health inequalities** • **Refugee status**	Data sources: PowerPoint Presentation (datawand.info) The bar charts below show household deprivation, economic activity, and levels of unpaid care in Wandsworth. Wandsworth has a higher proportion of deprived households compared to Inner London, London or England. A lower proportion of residents reported providing unpaid care compared to London and England, and Wandsworth had lower levels of economic inactivity compared to the rest of London. 



Census 2021 asked "Do you look after, or give any help or support to, anyone because they have long-term physical or mental health conditions or illnesses, or problems related to old age?". People were asked to exclude anything they did as part of their paid employment. This is different from the 2011 Census question, which began "Do you look after, or give any help or support to family members, friends, neighbours or others".

*ONS have used **Age-standardised proportions** as they enable comparisons between populations over time and across geographies, as they account for differences in the population size and age structure.



Socio-Economic Status - Mental Health Need

<u>Low Income</u>

In 2023 there were 18,250 children (0-19 years) living in poverty:

- Estimated 3,650 with a mental health disorder.

	Estimates below (2022) are based on age standardised prevalence rates (16-64 years old): • Estimated 2,249 unemployed women with a CMD. • Estimated 1,348 unemployed men with a CMD. • Estimated 2,838 benefit claimants with a CMD. <u>Carers</u> • It is estimated in 2022 that there are approximately 830 young carers In Wandsworth • Estimated 19,000 Wandsworth residents (16-90 yrs. old) identified as carers. • Estimated 3,000 residents (16-90 yrs. old) providing more than 20 hours informal care: Estimated 856 with a common mental health disorder. <u>Refuges and Asylum Seekers</u> As of July, 2025 there were 43 unaccompanied asylum-seeking children in Wandsworth: • Estimated 26 with a psychiatric disorder. • Estimated 22 with PTSD. In 2024 Wandsworth supported a total of 286 Asylum seekers. • Estimated 90 experiencing depression. • Estimated 30 experiencing anxiety. • Estimated 90 experiencing PTSD.

Data gaps

Data gap(s)	How will this be addressed?
Data on protected characteristics is available at Borough Level but is not always available for specific population groups in relation to strategic priorities.	Programme and service leads across Council-led priority areas to routinely implement the Mental Well-being Impact Assessment tool. Using the MWIA will identify risk and protective factors regarding new services, programmes and policies.
The MHNA identified a need to improve data collection across mental health services. The implementation of the PMHAP will include efforts to improve data collection and information related to protected groups and particular Global Majority groups where data was inconsistent.	This has already been identified as a priority through the MHNA recommendations and has been prioritised by both health and social care commissioners. This will be monitored through the Wandsworth Mental Health Partnership Group.

4. Impact

Protected group	Positive	Negative

Age	One of the PMHAP key priorities is to ensure that children are given the best start in life. Several programmes that have been identified will ensure that children and young people will receive evidenced based services to improve well-being for example: Family hubs and whole school approach to mental health and wellbeing. The PMHAP identifies a range of programmes to support working age adults and older people's mental health and wellbeing. Programmes included physical activity programmes these include but are not limited to employment and skills support, access to affordable homes and cost of living support.	The PMHAP sets out a clear vision and approach to improve the mental health and well-being of residents. The strategy is based upon up-to-date health intelligence, includes a both universal and targeted approaches to reduce the impact of health inequalities to those with protected characteristics. The PMHAP sets out to challenge racism, discrimination and stigma to provide an environment for people to flourish. The PMHAP will improve outcomes for marginalised communities. Monitoring implementation of the strategy will help ensure that any unintended consequences are noted and addressed.
Disability	The PMHAP has been informed by a comprehensive needs assessment and this EINA. This intelligence provides an overview of needs regarding disability. The strategy identifies a range of programmes and services to reduce inequalities that affect disabled people. These include programmes to reduce poverty, improve the environment, give children the best start in life and provide more equitable access to council services including social care and housing support.	As above
Sex	The PMHAP has been informed by a comprehensive needs assessment and this EINA. This intelligence provides an overview of needs regarding sex. The strategy identifies a range of programmes and services to reduce inequalities that affect both men and women. These include programmes to reduce violence against women and girls and suicide prevention programmes to address the disproportionate impact on middle aged men.	As above

Gender reassignment	The data included in this EINA identifies an increased risk of poor mental health for people who are transgender. The PMHAP identifies discrimination as a driver for poor mental health. Transgender people will benefit from the broad offer the council provides for residents through the equitable delivery of services and programmes. The council will work with health and social care providers across the system to highlight the inequalities faced and to prioritise services that support people with protected characteristics. The implementation of Mental Well-being Impact Assessments will provide a robust tool to identify the positive and negative impact of policies, programmes and services on residents. This in turn will be incorporated into the development of policies and programmes.	As above
Marriage and civil partnership	The PMHAP has been informed by a comprehensive needs assessment and this EINA. This intelligence provides an overview of needs regarding the Wandsworth population. The strategy includes both universal and targeted approaches to reduce the impact of health inequalities to groups with protected characteristics including through marriage and civil partnership. The strategy identifies a range of programmes and services to reduce inequalities that affect all residents. These include programmes to reduce poverty, improve the environment, give children the best start in life and provide more equitable access to council services including, cost of living, social care and housing support.	As above
Pregnancy and maternity	The PMHAP has been informed by a comprehensive needs assessment and this EINA. This intelligence provides an overview of	As above

	needs regarding pregnancy and maternity. The strategy recognises that there are specific challenges facing pregnant and post-natal women. The strategy identifies a range of programmes and services to reduce inequalities that affect this group. These include programmes to reduce poverty by providing employment skills, cost of living support and to improve the environment to ensure pregnancies can proceed in a health way and to ensure children get the best start in life. The strategy promotes approaches to reduce violence against women and girls.	
Race/ethnicity	The PMHAP has been informed by a comprehensive needs assessment and this EINA. This intelligence provides an overview of needs regarding the ethnic minority population in Wandsworth. The plan recognises that racism and discrimination contribute to poor mental health in these communities. The PMHAP supports the recommendations of the Director of Public Health's Annual Public Health Report, 'Enough is Enough', to develop anti-racist strategies, develop culturally competent community-based services and to train frontline staff in cultural competence. The plan also supports the development of the Patient and Carers Race Equality Framework (PCREF) across health and social care services. The PCREF should be delivered through genuine engagement with ethnic minority communities.	As above. The MHNA identified gaps in data regarding referrals and treatment for ethnic groups. There is currently work led by the mental health trust to improve data collection through the trust Health Inequalities Strategy and the PCREF, this will improve data collection. Public Health are part of the Health Inequalities Strategy Steering Group and will continue to advocate for more complete data.
Religion and belief, including non belief	The PMHAP has been informed by a comprehensive needs assessment and this EINA. This intelligence provides an overview of needs regarding the Wandsworth population. The strategy includes both universal and targeted	As above

	approaches across the life-course to reduce the impact of health inequalities to groups with protected characteristics including those with and without religious beliefs. The strategy identifies a range of programmes and services to reduce inequalities that affect all residents. These include programmes to reduce poverty, improve the environment, give children the best start in life and provide more equitable access to council services including, cost of living, social care and housing support.	
Sexual orientation	The data included in this EINA identifies an increased risk of poor mental health of people who identify as LGBTQ+. The PMHAP identifies discrimination as a driver for poor mental health. LGBTQ+ communities will benefit from the broad offer the council provides for residents through the equitable delivery of services and programmes. The council will work with health and social care providers across the system to highlight the inequalities faced and to prioritise services that support people with protected characteristics. The implementation of Mental Well-being Impact Assessments will provide a robust tool to identify the positive and negative impact of policies, programmes and services on residents. This in turn will be incorporated into the development of future policies and programmes.	As above

Socio-economic status (to be treated as a protected characteristic under Section 1 of the Equality Act 2010) Include the following groups: • **Deprivation (measured by the 2019 English Indices of Deprivation)** • **Low-income groups & employment** • **Carers** • **Care experienced people** • **Single parents** • **Health inequalities** • **Refugee status**	One of the key priorities of the PMHAP is to reduce poverty. Poverty is a driver for poor mental health outcomes. The PMHAP identifies several programmes that will contribute to addressing socio-economic disparities. These include interventions associated with the cost-of-living programme as well as a commitment to pay the London Living Wage. Other programmes include advice services to support people experiencing debt and programmes to improve resident's ability to secure sustained employment.	As above

5. Actions to advance equality, diversity and inclusion

Action	Lead Officer	Deadline
Ensure programmes identified through the PMHAP are aware of the inequalities that groups with protected characteristics face and use logic-modelling to help identify key outcomes to reduce them.	Graeme Markwell	March 2026
Ensure that policies, strategies and programmes utilise the Mental Well-being Impact Assessment (MWIA) Tool to identify the positive and negative impact on residents' well-being.	Graeme Markwell	March 2026
Ensure that future EINAs relating to PMHAP programmes use the MWIA to inform the impact on groups with protected characteristics. This in turn will be incorporated into the development of council-led policies and programmes.	Graeme Markwell	March 2026
Ensure Public Health membership of the SWLSt.George's Mental Health Trust Health Inequalities Steering Group	Graeme Markwell	March 2026
Ensure that PMHAP contributes to the development and ongoing review of health inequalities data, specifically relating to the use of Trust services by individuals with protected characteristics.	Graeme Markwell	January 2027
To provide regular updates to Directorate Equality Groups across the council.	Graeme Markwell	March 2026